

hovid-Irbesartan Tablet 300 mg

VIIRBsa-PC6 (MY)

DESCRIPTION

Caplet-shaped, white to off-white, 16.00 mm long, 8.00 mm wide, film-coated tablet, bevel-edged with shallow convex faces, thickness of 5.45 - 5.65 mm and "HOVID" embossed on one face.

COMPOSITION

Each tablet contains: Irbesartan 300 mg

PHARMACODYNAMICS

Irbesartan is a potent, orally active, selective angiotensin-II receptor (type AT₁) antagonist. It is expected to block all actions of angiotensin-II mediated by the AT₁ receptor, regardless of the source or route of synthesis of angiotensin-II. The selective antagonism of the angiotensin-II (AT₁) receptors results in increases in plasma renin levels and angiotensin-II levels, and a decrease in plasma aldosterone concentration. Serum potassium levels are not significantly affected by irbesartan alone at the recommended doses. Irbesartan does not inhibit ACE (kininase-II), an enzyme which generates angiotensin-II and also degrades bradykinin into inactive metabolites. Irbesartan does not require metabolic activation for its activity.

PHARMACOKINETICS

Absorption: Irbesartan is rapidly absorbed from the gastrointestinal tract with an oral bioavailability of 60 to 80%. Peak plasma concentrations of irbesartan occur 1.5 to 2 hours after an oral dose. Irbesartan may be dosed without regards to meals

Distribution: Irbesartan is about 96% bound to plasma proteins.

The volume of distribution is 53 - 93 litres.

Metabolism: Irbesartan is metabolised in the liver via glucuronide conjugation and oxidation, primarily by the cytochrome P450 isoenzyme CYP2C9, to inactive metabolites. The major circulating metabolite is irbesartan glucuronide (approximately 6%).

Elimination: It is excreted as unchanged drug and metabolites in the bile and in urine. The terminal elimination half-life is about 11 to 15 hours. The total body clearance is 157 - 176 ml/min.

INDICATION

- Treatment of essential hypertension.
- Treatment of diabetic nephropathy with an elevated serum creatinine and proteinuria (>300mg / day) in patients with type 2 diabetes and hypertension. In this population, Irbesartan reduces the rate of progression of nephropathy as measured by the occurrence of doubling of serum creatinine or end-stage renal disease (need for dialysis or renal transplantation)

CONTRAINDICATIONS

Hypersensitivity to the active substance, or to any of the excipients.

Second and third trimesters of pregnancy.

The concomitant use of Irbesartan with aliskiren-containing products is contraindicated in patients with diabetes mellitus or renal impairment (glomerular filtration rate (GFR) <60 ml/min/1.73m²).

WARNINGS AND PRECAUTIONS

Intravascular volume depletion: symptomatic hypotension, especially after the first dose, may occur in patients who are volume and/or sodium depleted by vigorous diuretic therapy, dietary salt restriction, diarrhoea or vomiting. Such conditions should be corrected before the administration of Irbesartan.

Renovascular hypertension: there is an increased risk of severe hypotension and renal insufficiency when patients with bilateral renal artery stenosis or stenosis of the artery to a single functioning kidney are treated with medicinal products that affect the renin-angiotensin-aldosterone system. While this is not documented with Irbesartan, a similar effect should be anticipated with angiotensin-II receptor antagonists.

Renal impairment and kidney transplantation: when Irbesartan is used in patients with impaired renal function, a periodic monitoring of potassium and creatinine serum levels is recommended. There is no experience regarding the administration of Irbesartan in patients with a recent kidney transplantation.

Hypertensive patients with type 2 diabetes and renal disease: the effects of irbesartan both on renal and cardiovascular events were not uniform across all subgroups, in an analysis carried out in the study with patients with advanced renal disease. In particular, they appeared less favourable in women and non-white subjects.

Dual blockade of the renin-angiotensin-aldosterone system (RAAS): There is evidence that the concomitant use of ACE-inhibitors, angiotensin II receptor blockers or aliskiren increases the risk of hypotension, hyperkalaemia and decreased renal function (including acute renal failure). Dual blockade of RAAS through the combined use of ACE-inhibitors, angiotensin II receptor blockers or aliskiren is therefore not recommended. If dual blockade therapy is considered absolutely necessary, this should only occur under specialist supervision and subject to frequent close monitoring of renal function, electrolytes and blood pressure. ACE-inhibitors and angiotensin II receptor blockers should not be used concomitantly in patients with diabetic nephropathy.

Hyperkalaemia: as with other medicinal products that affect the renin-angiotensin-aldosterone system, hyperkalaemia may occur during the treatment with Irbesartan, especially in the presence of renal impairment, overt proteinuria due to diabetic renal disease, and/or heart failure. Close monitoring of serum potassium in patients at risk is recommended.

Lithium: the combination of lithium and Irbesartan is not recommended.

Aortic and mitral valve stenosis, obstructive hypertrophic cardiomyopathy: as with other vasodilators, special caution is indicated in patients suffering from aortic or mitral stenosis, or obstructive hypertrophic cardiomyopathy.

Primary aldosteronism: patients with primary aldosteronism generally will not respond to antihypertensive medicinal products acting through inhibition of the renin-angiotensin system. Therefore, the use of Irbesartan is not recommended.

General: in patients whose vascular tone and renal function depend predominantly on the activity of the renin-angiotensin-aldosterone system (e.g. patients with severe congestive heart failure or underlying renal disease, including renal artery stenosis), treatment with angiotensin converting enzyme inhibitors or angiotensin-II receptor antagonists that affect this system has been associated with acute hypotension, azotaemia, oliguria, or rarely acute renal failure. As with any antihypertensive agent, excessive blood pressure decrease in patients with ischaemic cardiopathy or ischaemic cardiovascular disease could result in a myocardial infarction or stroke. As observed for angiotensin converting enzyme inhibitors, irbesartan and the other angiotensin antagonists are apparently less effective in lowering blood pressure in black people than in non-blacks, possibly because of higher prevalence of low-renin states in the black hypertensive population.

Pregnancy: Angiotensin II Receptor Antagonists (AIIRAs) should not be initiated during pregnancy. Unless continued AIIRA therapy is considered essential, patients planning pregnancy should be changed to alternative antihypertensive treatments which have an established safety profile for use in pregnancy. When pregnancy is diagnosed, treatment with AIIRAs should be stopped immediately, and, if appropriate, alternative therapy should be started.

Lactose: this medicinal product contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicinal product.

Paediatric population: current data are insufficient to support an extension of the use in children.

PREGNANCY AND LACTATION

Pregnancy: It is not recommended during the first trimester of pregnancy and is contraindicated during the second and third trimesters of pregnancy

Lactation: Available evidence and/or expert consensus is inconclusive or is inadequate for determining infant risk when used during breastfeeding. Weigh the potential benefits of drug treatment against potential risks before prescribing this drug during breastfeeding.

DRUG INTERACTIONS

Diuretics and other antihypertensive agents: Other antihypertensive agents may increase the hypotensive effects of irbesartan; however Irbesartan has been safely administered with other antihypertensive agents, such as beta-blockers, long-acting calcium channel blockers, and thiazide diuretics. Prior treatment with high dose diuretics may result in volume depletion and a risk of hypotension when initiating therapy with Irbesartan.

Aliskiren-containing products or ACE-inhibitors: dual blockade of the renin-angiotensin-aldosterone system (RAAS) through the combined use of ACE-inhibitors, angiotensin II receptor blockers or aliskiren is associated with a higher frequency of adverse events such as hypotension, hyperkalaemia and decreased renal function (including acute renal failure) compared to the use of a single RAAS-acting agent.

Potassium supplements and potassium-sparing diuretics: based on experience with the use of other medicinal products that affect the renin-angiotensin system, concomitant use of potassium-sparing diuretics, potassium supplements, salt substitutes containing potassium or other medicinal products that may increase serum potassium levels (e.g. heparin) may lead to increases in serum potassium and is, therefore, not recommended (see Special warnings and precautions for use).

Lithium: reversible increases in serum lithium concentrations and toxicity have been reported during concomitant administration of lithium with angiotensin converting enzyme inhibitors. Similar effects have been very rarely reported with irbesartan so far. Therefore, this combination is not recommended. If the combination proves necessary, careful monitoring of serum lithium levels is recommended.

Non-steroidal anti-inflammatory drugs: when angiotensin II antagonists are administered simultaneously with non-steroidal anti-inflammatory drugs (i.e. selective COX-2 inhibitors, acetylsalicylic acid (> 3 g/day) and non-selective NSAIDs), attenuation of the antihypertensive effect may occur. As with ACE inhibitors, concomitant use of angiotensin II antagonists and NSAIDs may lead to an increased risk of worsening of renal function, including possible acute renal failure, and an increase in serum potassium, especially in patients with poor pre-existing renal function. The combination should be administered with caution, especially in the elderly. Patients should be adequately hydrated and consideration should be given to monitoring renal function after initiation of concomitant therapy, and periodically thereafter.

Additional information on irbesartan interactions: the pharmacokinetic of irbesartan is not affected by hydrochlorothiazide. Irbesartan is mainly metabolised by CYP2C9 and to a lesser extent by glucuronidation. No significant pharmacokinetic or pharmacodynamic interactions were observed when irbesartan was coadministered with warfarin, a medicinal product metabolised by CYP2C9. The effects of CYP2C9 inducers such as rifampicin on the pharmacokinetic of irbesartan have not been evaluated. The pharmacokinetic of digoxin was not altered by coadministration of irbesartan.

MAIN SIDE/ADVERSE EFFECTS

Investigations: hyperkalaemia, increases in plasma creatine kinase

Cardiac disorders: tachycardia.

Vascular disorders: orthostatic hypotension, flushing.

Nervous system disorders: headache, dizziness.

Respiratory, thoracic and mediastinal disorders: cough, upper respiratory infection.

Gastrointestinal disorders: nausea, vomiting, diarrhoea, dyspepsia/heartburn, dysgeusia.

General disorders and administration site conditions: fatigue.

Renal and urinary disorders: impaired renal function including cases of renal failure in patients at risk.

Skin and subcutaneous tissue disorders: urticaria, leukocytoclastic vasculitis.

Musculoskeletal and connective tissue disorders: arthralgia, myalgia (in some cases associated with increased plasma creatine kinase levels), rhabdomyolysis, muscle cramps.

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Immune system disorders: hypersensitivity reactions such as angioedema, rash, urticaria.

Hepato-biliary disorders: hepatitis, abnormal liver function, cholestasis, jaundice.

Hematologic disorders: thrombocytopenia.

Reproductive system and breast disorders: sexual dysfunction.

Ear and labyrinth disorders: tinnitus

OVERDOSE AND TREATMENT

Symptoms: The most likely manifestations of overdose are expected to be hypotension and tachycardia; bradycardia might also occur from overdose.

Treatment: No specific information is available on the treatment of overdose with Irbesartan. The patient should be closely monitored, and the treatment should be symptomatic and supportive. Suggested measures include induction of emesis and/or gastric lavage. Activated charcoal may be useful in the treatment of overdose. Irbesartan is not removed by haemodialysis.

DOSAGE AND ADMINISTRATION

Oral

The usual recommended initial and maintenance dose is 150 mg once daily, with or without food.

hovid-Irbesartan is available in strength of 300mg. It is not able to deliver all approved dose regimens of Irbesartan; other approved dosage forms and strengths of Irbesartan should be used in such cases.

Irbesartan at a dose of 150 mg once daily generally provides a better 24 hour blood pressure control than 75 mg. However, initiation of therapy with 75 mg could be considered, particularly in haemodialysed patients and in the elderly over 75 years.

In patients insufficiently controlled with 150 mg once daily, the dose of Irbesartan can be increased to 300 mg, or other antihypertensive agents can be added. In particular, the addition of a diuretic such as hydrochlorothiazide has been shown to have an additive effect with Irbesartan

In hypertensive type 2 diabetic patients, therapy should be initiated at 150 mg irbesartan once daily and titrated up to 300 mg once daily as the preferred maintenance dose for treatment of renal disease.

Renal impairment: no dosage adjustment is necessary in patients with impaired renal function. A lower starting dose (75 mg) should be considered for patients undergoing haemodialysis

Hepatic impairment: no dosage adjustment is necessary in patients with mild to moderate hepatic impairment. There is no clinical experience in patients with severe hepatic impairment.

Elderly patients: although consideration should be given to initiating therapy with 75 mg in patients over 75 years of age, dosage adjustment is not usually necessary for the elderly.

Paediatric patients: irbesartan is not recommended for use in children and adolescents due to insufficient data on safety and efficacy.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:
Store below 30°C.

Presentation / Packing:
Blister pack of 10's, 30's and 100's.

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